

INFANT AND TODDLER EDUCATION AND CARE



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1.1: Welcome to Infant and Toddler Education and Care

Infant and Toddler Education and Care. ECPK 324 is an introduction to the field of infant and toddler childcare. Through reading the materials in the book and by participating in activities you can expect to explore research-based child learning and development; developmentally appropriate activities using a strength based approach; quality caregiving routines; and environmental influences on growth and development. Topics discussed in the text include:

- Research on brain development
- Children with special needs and their families
- Interrelated nature of development across domains (social, emotional, language, cognitive, physical)
- Dual language learners
- Family partnerships
- Culturally sensitive care

Infant and Toddler Education and Care are: *Introduction to Early Childhood Education; Child Development; and Early Childhood Learner with Special Needs*. BSU students also need to be admitted to the Early Education and Care professional education program.

Language Development & Early Literacy; Infant and Toddler Education & Care; The Creative Arts & Curriculum Design in Early Childhood; Childcare Programming, Administration, & Supervision; and Mentored Teaching Fieldwork and Student Teaching. Recent presentations include: An Early Childhood Bachelor's Degree Program that Authentically Prepares the Birth to Grade-3 Workforce; Creating a Video Explanation to Improve Teaching & Learning; and Preparing Faculty to Effectively Mentor Field Experiences.

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1.2: Toddler Development

Learning Outcome

explain and **recognize** stages and milestones in physical, social, emotional, sensory, linguistic, and cognitive development for younger toddlers from 12 to 24 months and for older toddlers from 22 to 36 months. The outcome supports the NAEYC *Professional Standards and Competencies for Early Childhood Educators*:

Introduction

7 Toddler Development Facts you Probably Didn't know by Jeff Hayward (2016).

1. Toddlers are often nicer to strangers than adults presumably for safety until trust is established, while adults lose tolerance for personality traits they don't like
2. Toddlers' vocabulary grows faster than yours
3. Toddlers are born ambidextrous
4. A toddler in the most physically active phase of their life
5. A toddler's hearing is super-sensitive
6. Brain activity reaches a life-time high during toddler years
7. Frequent colds may hinder physical growth in toddlers

DAP in a Toddler Room

- [How to use developmentally appropriate practices in a Toddler Room](#)

Offer choices to encourage independence and autonomy.

Initiate conversations to build vocabulary and communication skills. In conversations with toddlers, always attempt to convey meaning clearly (with short, direct sentences), and include appropriately challenging words to support the explosion of a toddler's vocabulary.

Use positive guidance techniques such as careful supervision, anticipating likely behaviors, positive encouragement and feedback, and redirecting a toddler's attention to acceptable activities.

Prepare the environment by providing duplicate equipment, checking to ensure safety, and providing props that encourage toddlers to explore primary emotions. Create indoor and outdoor spaces for flexibility, safe sensory exploration, and physical development.

Work with families. Establish open and honest communication with toddlers' families. Cooperation and coordination with families will provide toddlers with an added level of security and consistency as they dance between autonomy and independence.

Texas Child Care Quarterly, 37, (3). Available at: www.childcarequarterly.com/pdf/winter13_dap.pdf

How can I learn more about developmental milestones?

Massachusetts Early Learning Guidelines for Infants and Toddlers you will notice that the learning guidelines are presented in table format. For each Guideline the indicators are divided into two sections: a) young toddler and b) older toddler. The indicators describe **expected** observable behaviors or skills of children which are developmental milestones.

<https://www.zerotothree.org/resources/series/your-child-s-development-age-based-tips-from-birth-to-36-months>

- [What are the developmental considerations for toddler care?](#)
 - Mobile younger toddlers (12 to 18 months) are eager to **explore**.
 - Toddlers (16 to 36 months) are working on their **identity**; they want to know who they are and who's in charge.
- Lally, Torres, and Phelps (2010) list six key components of early group experience essential to toddler development: group size; quality of the physical environment; primary caregiving assignments; continuity of care; cultural and familial continuity; and meeting the needs of the individual within the group context.

Group Size

Age Range for Toddlers	Group size = 6	Group size= 8	Group size = 10	Group size = 12
12 to 28 months	1:3	1:4	1:4	1:4
21 to 36 months		1:4	1:5	1:6

MA licensing regulations have requirements for programs for group size and ratio. The regulations state:

1. All group assignments must be developmentally appropriate for the individual child.
2. The group size must be appropriate for the activities planned and for the characteristics of children assigned to the group.
3. Each child must be assigned to a consistent group with consistent staffing.
4. Children may be assigned to fixed age groups outside their chronologically defined age group based on a review of the child's most recent progress report or a narrative from the child's parent addressing the child's abilities in the areas of mobility, fine and gross motor control, communication, social interactions, and cognition.

Fixed Age Groups.

Regulation Number	Age Group / Full or Half Day Program	Maximum Group Size	Educator / Child Ratio	Educator Qualifications
7.10(9)(b)1	Infants (up to 15 months old) (Full or Half Day)	7	1: 3, 1 additional educator for 4 – 7 infants	At least 1 Infant/Toddler Teacher, per 7.09(18)(c)2
7.10(9)(b)2	Toddlers (15 to 33 months) (Full or Half Day)	9	1:4, 1 additional educator for 5 – 9 toddlers	At least 1 Infant/Toddler Teacher, per 7.09(18)(c)2

Regulation Number	Age Group	Maximum Group Size	Educator / Child Ratio	Educator Qualifications
7.10(9)(c)1	Infant / Toddler Group	9; no more than 3 infants	1:3; 1 additional educator for 4 – 9 children	At least 1 Infant/Toddler Teacher, per 7.09(18)(c)2
7.10(9)(c)2	Toddler / Preschool Group	9	1:5; 1 additional educator for 6 – 9 children	At least 1 Infant/Toddler and Preschool Teacher, per 7.09(18)(c)2

The Environment

Safety — Are all the electrical outlets covered? Is there adequate padding under climbing equipment in case of a child falling? Are the walkways clear and unobstructed? Is the play area fenced in? Are there first aid supplies available?

Health — Is the diapering area kept separate from the food preparation and feeding areas? Are the food preparation, feeding, and toileting areas cleaned and sanitized daily and after each use. Do the children get fresh air every day?

Comfort — Is there soft furniture accessible to the children? Are there safe, comfortable, adequate sleeping arrangements? Is there furniture that is both child-sized and adult-sized in the rooms? Can toddlers sit and stand as they choose?

Convenience — Is there easy access to sinks for adults and children? Are the materials and supplies stored near routines and activities?

Encourages Movement — Do toddlers have enough space to crawl, roll, pull up, walk, climb, run, jump and explore when both indoors and outdoors?

Allows for Choice — Are the toddlers offered choices at mealtimes and during activities?

Ensures accessibility — Are the toys out at the children's level? Can toddlers reach toys and materials independently? Are there spaces for both quiet and active play? Are there toys to represent imaginative play, constructive play, sensory play and literacy? Are toddlers allowed to play with sand, water, dress up clothes, etc.?

Educator Scheduling

Each child must be assigned to a consistent group with consistent staffing. The assignment of a primary educator means the child's family, the administrator, and educator know who is principally responsible for that child. Primary caregiving does not mean that one person cares for an infant or toddler exclusively, all of the time as there will be team teaching. Primary caregiving ensures that the toddler can build an intimate relationship with at least one educator.

The way it should be

Two weeks later: After his mother kissed him, Tim waved good-bye and then he said, "Hi" again to Mindy, who ruffled his hair. Tim made a beeline for the block area, but when he got there Wong Chen had corralled all of the red blocks Tim wanted. Tim squatted down and watched Wong Chen. In a minute, Mindy came over and squatted down too, and they both watched him. Then Mindy said, "Tim likes those blocks too, Wong Chen; would you let him play?" Wong Chen looked at the floor, and then he looked up and frowned at Mindy and then at Tim. Mindy said, "OK, Wong Chen, then Tim can use them later." And then to Tim, while pointing to some bristle blocks, "Maybe you'd like these? I don't think anyone has those." As Tim started to walk away, Wong Chen handed him one red block and Mindy said, "Thank you, Wong Chen, we'll use that."

Continuity of Care

Looping and Attachment in Early Childhood Education: How the Applications of Epigenetics Demand a Change. *Journal of the Scholarship of Teaching and Learning*, 17, (2), 85-100. doi: 10.14434/josotl.v17i2.20840

What do you think about the practice of looping? How might it work in a childcare program you observed or worked in?

5. Cultural and Familial Continuity

What are your experiences with culturally representative educators in the programs you observed in or worked in?

6. Meeting the Needs of the Individual in a Group

Development in the Domains

Social-Emotional Development

Massachusetts early learning guidelines for infants and toddlers (2010), define social development as the child's ability to relate to and interact with people. Toddlers are scientists, experimenting with interactions that occur between themselves and the people around them. Be mindful that you are a role model of behavior and culture for the toddler.

- relate to, trust, and develop an attachment to caregivers.
- interact with peers.
- experience and express a range of emotions.
- self-regulation
- the positive sense of self
- initiative

REMEMBER

- Social and emotional development impacts all other areas of development and is the foundation for all future development.
- Toddlers will carry inside the impact of the relationships with caregivers at this stage of life; setting the foundation for school readiness and lifelong success. When caregivers are loving, responsive and consistent, toddlers learn they are valued and the world is primarily satisfying and predictable.
- Research shows that children who have healthy social and emotional skills tend to learn better, are more likely to stay in school, and will be better able to make and keep lifelong friends.

REFLECT

- **How will you promote secure relationships with toddlers?**
- **How will you encourage toddlers to experience, regulate, and express emotions?**

For further information

Your Child's Cues from Birth to Age 2 available at http://csefel.vanderbilt.edu/documents/reading_cues.pdf

Language and Communication Development

REMEMBER

- An understanding of spoken (or signed) language (receptive language)
- Use of expressive language
- Participation in social communication
- Asking questions and telling stories (narratives)
- Increased ability to produce sounds accurately and put words together to make meaning (phonology and syntax)
- An interest in books and listening to stories
- Beginning to use writing tools.

For further information

Read Fuligni, A. S., Hoff, E., Zepeda, M., & Mangione, P. (March 2014). [Development of infants and toddlers who are dual language learners](#). Working paper #2. Center for Early Care and Education Research-Dual Language Learners (CECER-DLL). Chapel Hill: The University of North Carolina, Frank Porter Graham Child Development Institute.

<http://www.cdc.gov/ncbddd/watchmetra...g/module2.html>)

- **Key Takeaway – Toddler's Cognitive Development**
- **toddlers think in fundamentally different ways from older children and adults.** Toddlers' brains are partially developed. As a result, toddlers think in fundamentally different ways from adults. Interpreting toddler behavior according to adult logic is pointless – and it sometimes leads us to draw conclusions that aren't always helpful. Toddlers make much more sense if you step inside their heads and try and see the world from their level of development.

[Anita Cleare](#), a Parenting specialist, provides an insight into how toddlers' brains work so that we can interpret toddler behavior accurately. Here are some scenarios to further explain toddler thinking.

- Educator: "The purpose of this walk is to get to the park."
- Educator thinks: **Why can't she walk from A to B without being distracted by a leaf?**
- Toddler: "Wow, look at that shiny wrapper blowing along the ground!"
- Educator: "Stop being so obsessive, the blue cup is in the dishwasher."
- Educator thinks: **Why isn't the green cup as good as the blue cup?**
- Toddler: "No green!."
- Educator: "No thank you, you can't take toys from the other children."
- Educator thinks: **Why can't toddlers share?**
- Toddler: "It's mine."
- Educator: "You got hurt, there is no blood so no Band-Aid, I'll kiss your boo-boo."
- Educator thinks: **What a drama queen!**
- Toddler: "Need a Band-Aid!!"
- The toddler thinks: Help! I'm broken! Fix me!
- Educator: "It's time to get your jacket on so we can go outdoors."
- Educator thinks: We only have 20 minutes left of outdoor time, by the time she finishes outdoor time is over
- Toddler: **"Me do it!"**
- The toddler thinks: I know I can do it!

To learn more:

Texas Child Care Quarterly, 37, (4) available at: www.childcarequarterly.com/pdf/spring14_infants.pdf

ScienceDaily. Retrieved August 3, 2017 from www.sciencedaily.com/releases/2007/10/071017174331.htm

Physical Health and Well Being

- Motor development (large and small muscles or gross and fine motor)
- Use of the senses to guide and integrate interactions
- Skills for healthy life practices
 - Eating
 - Sleeping
 - Toileting
 - Hygiene practices

ncchildcare.dhhs.state.nc.us/pdf_forms/dcd_infant_toddler_health_physical_development.pdf

Nutrition

Young toddlers may begin to:

- Want to feed themselves.
- Eagerly participate in snacks and mealtimes.
- Bite, chew and swallow soft food smoothly.
- Show interest in many types of food and no interest in other foods.
- Eat inconsistently (eat a lot at one meal and little at the next, be too busy playing to eat).
- Ask for food when hungry or accept food when offered.
- Be willing to try new foods.

Older toddlers may begin to:

- Enjoy helping with meal and snack routines (set table, wash hands, throw away trash).
- Bite and chew solid food more easily.
- Accept or refuse food depending on their appetite and interest.
- Notice and talk about food textures, temperatures, and tastes (crunchy crackers, warm soup, sweet apples).
- Understand that some foods are good for them (fresh fruits, vegetables, milk) and some are not very healthy (potato chips, soda).

Sleep

Young toddlers may begin to:

- Show they know when it is time to sleep (points at the bed, get blanket).
- Cooperate with sleep routines (choose a book, get preferred sleep toy).
- Use simple sounds, gestures, or words to show they are tired.

Older toddlers may begin to:

- Use words for being tired.
- Initiate and participate in sleep routines (wash hands after lunch, get blanket, lie down on bed or mat).
- Fall asleep on their own.

Physical Activity

Young toddlers may begin to:

- Anticipate and ask for outdoor play (points at door and say “Out!”; resist coming indoors).
- Engage in regular and sustained movement (rides toy all around the play yard, go up and down the slide over and over).
- Develop strength and stamina as they use large muscles and participate in physical activity for longer periods of time.
- Enjoy active play and seek to be physically active (choose to play often on the climber, laugh and squeal while running).

Older toddlers may begin to:

- Engage in lively movements by choice for long periods of time indoors and outdoors.
- Enjoy more complex movement activities (running, jumping, and skipping).
- Match body movements to rhythm (move slowly to slow music, dance in time with the music).
- Show pride in new skills and strengths (ask others to watch them, say “I’m big and strong!”).

Self Care

- Use simple sign language, facial expressions, sounds or words to tell you what they need.
- Cooperate and help with care routines (mouth-care, hand-washing, diapering, dressing, bathing).
- Drink from a cup and feed themselves with their fingers or a spoon.
- Protect personal objects and space from others.
- Help with clean-up routines.
- Show excitement at completing self-care tasks (show teeth after brushing, hold up hands after washing).
- Cooperate with medical care, positioning, and use of adaptive equipment.
- Identify and use objects and follow routines that are comforting (get their blanket and lie down where they usually sleep, pick out a favorite books to be read before lunch).

Older toddlers may begin to:

- Use words or sign language to ask for the things they need (food when hungry, drink when thirsty, go outdoors when they need to be physically active).
- Soothe themselves when needed (find a quiet area for alone time, look at a book before nap).
- Increase independence with basic self-help skills (pull up pants, put on socks, shoes, and hat).
- Remember and imitate details of self-care routines (talk through steps while washing hands; tell doll how to brush teeth during pretend play).
- Start self-care routines and complete some steps independently (undressing, hand washing, brushing teeth).
- Understand the role of people who help children stay healthy (doctors, nurses, dentists).
- Show appreciation for possessions (put toys away, handle materials carefully).
- Use adaptive equipment, ask for help with positioning and movement, or participate in medical care routines as needed.

Safety Awareness

Young toddlers may begin to:

- Experience cause and effect (going downhill fast cause falls; turning the TV up too loud hurts ears).
- Show some caution on uneven ground and heights.
- Notice and imitate adult reactions to dangerous people and situations.
- Respond to warnings and directions from others.
- Understand the difference between what should be eaten and what should not.

Older toddlers may begin to:

- Remember cause and effect experiences and apply their experiences to future situations (avoid touching cold railing, walk slowly downhill where fall happened).
- Increase self-control over their impulses.
- Recognize and avoid situations that might be unsafe.
- Understand what their bodies can do and understand their limits.
- Watch for adult reactions to unfamiliar things or situations that might be dangerous.
- Understand and follow basic health and safety rules. They still require close supervision from caregivers to follow these health and safety rules consistently.
- Feel proud when they follow safety rules and ashamed when they do not (say, "Look, I waited!" at a corner; hang head after trying to reach forbidden item).

Health

[poisoning](#), not smoking around children, using sunscreen, and gun safety.

Sleep problems and behavioral challenges are very common at this age and can be frustrating for caregivers. I invite you to look at the [MedlinePlus](#) web site to explore resources related to the health of toddlers.

Key Takeaway

*It took me many years to see **caring as the core** of curriculum excellence. The deep assumption about caring is that it is something anyone can do, but we do not take care of human beings the same way we take care of a house or a lawn. In childhood, the sensations of the body are the pathways to the child's intellect and emotions. Caring routines involve engagement around bodily*

functions (elimination, cleaning, eating, sleeping) and therefore they hold the most intimate importance. The way we touch children increases or diminishes their self-worth. Our care of children's bodies is directly connected to the care of their minds.

Exchange magazine

 Phases Of Early Childhood Development

Infographic to illustrate early childhood development

Visually.

Remember

- A 28-month-old can reliably remember events, even up to 3 months after they happen (Fivush & Hamond, 1989). The components of memory require different abilities: the ability to hold objects and people in mind even when they cannot be seen (object permanence), to keep recent information in mind in order to perform a task (working memory), and to remember events from the past (long-term memory).
- Persistence is the ability to work toward a specific goal despite challenges or feelings of frustration.
- Problem-solving is how we figure things out. Remember that toddlers usually create their own “problems” to solve with the goals they have in mind.

Reflect

- Remember a time when you learned something new. **How did memory, persistence, and problem-solving help you learn?**
- Persistence and problem-solving skills can vary widely from child to child depending on temperament. **How do you support children who have different natural abilities in these areas? How might temperament affect “persistence through frustration?”**

References

Journal of Experimental Child Psychology, 47(2), 259-273

Read and Participate

1. Read Massachusetts Early Learning Guidelines for Infants and Toddlers Section III: Toddlers (12 months to 33 months) pages 85-159.
2. Complete the reading reflection using information from this chapter and the Early Learning Guidelines. Use your responses from the reading reflection to participate in the Blackboard or face-to-face class discussion.

Assignment

Supporting Infant and Toddler Development; Screening, and Assessment. To get an overview of the Ages and Stages Questionnaire, watch a 9-minute YouTube Video by Eagle Media produced in 2015. The video is titled *You & Your Young Child: Ages and Stages Questionnaire*

To complete the assignment:

Toddler ASQ Requirements

1. Locate a toddler from 15 to 33 months.
2. Complete the appropriate age group for ASQ 3 and ASQ 2SE available on Blackboard.
3. Use the correct questionnaire and scoring sheets for a toddler at 16, 18, 20, 22, 24, 27, 30 OR 33 months.
4. After completing the questionnaire, **write a paper** where you will answer the following questions:
 1. Describe a typical child of the age you observed and how the child you assessed is typical, developing above age level, or if there are concerns about development.
 2. What are the factors that affect this toddler's development?
 3. What is the child's learning and interaction style?
 4. What learning experiences might support this child's unique strengths and characteristics?
 5. Insert photos of the ASQ forms.

ASQ Grading Rubric and Sample Paper available on Blackboard.

References

<https://www.zerotothree.org/resource...lers-in-groups>

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1.3: Developmental Influences- Culture

Learning Objective

Explain how culture influences growth and development This meets the NAEYC Standard 1b, 6b and the MA Core Competency 1.A.2 @ Initial & 8.D. 12@Initial level.

INTRODUCTION

CULTURE

Cultural Humility: People, Principles, and Practices. to get you started thinking about the next assignment and to frame your thinking as you read this chapter. The 2012 video is by San Francisco State professor Vivian Chávez. She mixes poetry with music, interviews, archival footage, and images of community, nature and dance to explain what Cultural Humility is and why we need it. Cultural Humility is now used in public health, social work, education, and non-profit management. It is a daily practice for people who deal with hierarchical relationships, changing organizational policy and building relationships based on trust.^[1]

The Youngest Americans: A Statistical Portrait of Infants and Toddlers in the United States explore America's 12 million infants and toddlers and their demographic transformation in the U.S. **What did you learn about the ways you might differ in terms of race/ethnicity, country of origin, language, and family type. What did you learn about unfairness? What practices might be harmful to infants and toddlers? How do national policies support or harm infants and toddlers?**

INFANT AND TODDLER TEACHERS AS TRANSMITTERS OF CULTURE

Test, J.E. Infant and toddler teachers as transmitters of culture. *IJEC* **38**, 47–63 (2006). <https://doi.org/10.1007/BF03165977>

SUMMARY

CONCEPTUALIZING CULTURAL TRANSMISSION

- ARE INFANTS AND TODDLERS PART OF THIS PROCESS?
- TRANSMISSION OF CULTURE IN EARLY CHILDHOOD SETTINGS
- BACKGROUND AND OBSERVATIONS
- CONFLICTS OVER TOYS
- MEAL AND SNACK TIMES
- STRUCTURE OF GROUPS
- SPACE TO BE ALONE
- STIMULATION AND ACTIVITY LEVEL

THE ROLE OF CHILD CARE IN SOCIETY

- CONCLUSIONS
- #### REFERENCES

- Bakeman, R. & Brownlee, J. R. (1982). Social rules governing object conflicts in toddlers and preschoolers. In K. H. Rubin & H. S. Ross (Eds.), *Peer relationships and social skills in childhood* (pp. 99-111). New York: Springer-Verlag.
- <http://www.oecd.org/dataoecd/52/31/2534972.pdf>
www.sweden.se/templates/FactSheet___4132.asp
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1.4: Developmental Influences- Nutrition

Learning Objectives

- The objective in this section is for you to explain how nutrition influences growth and development This meets the NAEYC Standard 1b, 6b and the MA Core Competency 1.A.2 @ Initial & 8.D. 12@Initial level.

Interesting Facts

- Models for healthy behavior are early educators as 75 % of children spend time in child care.
- Children who are overweight or obese are at increased risk for health and socio-emotional problems.
- Being overweight in the preschool years is highly predictive of being overweight later in childhood.
- Overweight children are more likely than their peers to develop cardiovascular disease, type-2 diabetes, liver disease, sleep apnea, high cholesterol, and asthma.
- Food insecurity is associated with children's greater risk for being overweight.
- Shared mealtimes are associated with a number of benefits such as increased vocabulary and exposure to a wider variety of foods that may result in healthier eating habits.
- Research suggests that having a television on during mealtimes may promote unhealthy eating.

Developmental Considerations Infants

Infant Nutrition and Feeding: A Guide for Use in the WIC and CSF Programs (2009) The guide provides nutrition education to help you inform families of infants.

Stanford Children's Health website . The Infant Feeding Guide offers guidance for appropriate and healthy feeding of an infant during the first year of life. Here are tips from their website:

- Breast milk or formula provides your baby all the nutrients that are needed to grow.
- Your baby isn't physically developed enough to eat solid food from a spoon.
- Feeding your baby solid food too early may lead to overfeeding and being overweight.
- As a general rule, solid foods don't help babies sleep through the night.

Guide for formula feeding (0 to 5 months)

Age	Amount of formula per feeding	Number of feedings per 24 Hours
1 month	2 to 4 ounces	6 to 8 times
2 months	5 to 6 ounces	5 to 6 times
3 to 5 months	6 to 7 ounces	5 to 6 times

Feeding tips for infants after 4-6 months

- When starting solid foods, offer one new food at a time—not mixtures like cereal and fruit or meat dinners. Give the new food for two to three days before adding another new food. This way you can tell what foods an infant may be allergic to or can't tolerate.
- Begin with small amounts of new solid foods—a teaspoon at first and slowly increase to a tablespoon.
- There are no strict rules about the order in which you should give different foods in. Many people start with an infant cereal and gradually add fruits, vegetables, and proteins.
- Don't use salt or sugar when making homemade baby foods. Also, avoid feeding homemade spinach, beets, green beans, squash, and carrots for infants younger than age 6 months because of the risk for methemoglobinemia, a blood disorder that can interfere with oxygen delivery in the blood, due to high concentration of nitrates. Canned foods may contain large amounts of salt and sugar and shouldn't be used for baby food. Always wash and peel fruits and vegetables and remove seeds or pits. Take special care with fruits and vegetables that come into contact with the ground. They may contain botulism spores that cause food poisoning.
- Cow's milk shouldn't be added to the diet until the baby is age 12 months. Cow's milk doesn't provide the right nutrients for an infant.

- Limit fruit juice without sugar to a maximum of 4 to 6 ounces daily when the infant is able to drink from a cup (around age 6 months or older). Whole fruits and vegetables are a much healthier option.
- Feed all foods with a spoon. Your baby needs to learn to eat from a spoon. Only formula and water should go into the bottle.
- Expect a smaller and pickier appetite as the baby's growth rate slows around age 1.
- Once an infant is taking solids, offer sips of water.
- Offer a wide variety of foods early to develop good eating habits later.

Toddlers

North Carolina Division of Child Development: Infant Toddler Foundations (2008). The list is useful in understand what to expect from toddlers

Young toddlers may begin to:

- Want to feed themselves.
- Eagerly participate in snacks and mealtimes.
- Bite, chew, and swallow soft food smoothly.
- Show interest in many types of food and no interest in other foods.
- Eat inconsistently (eat a lot at one meal and little at the next, be too busy playing to eat).
- Ask for food when hungry or accept food when offered.
- Be willing to try new foods.

Older toddlers may begin to:

- Enjoy helping with meal and snack routines (set table, wash hands, throw away trash).
- Bite and chew solid food more easily.
- Accept or refuse food depending on their appetite and interest.
- Notice and talk about food textures, temperatures, and tastes (crunchy crackers, warm soup, sweet apples).
- Understand that some foods are good for them (fresh fruits, vegetables, milk) and some are not very healthy (potato chips, soda).

A resource to review

Bright Futures: Nutrition, 3rd Edition. Washington, DC: Author. The introduction provides information on promoting good nutrition and physical activity as essential components of a healthy lifestyle; establishing a framework for understanding how culture affects food choices and nutrition; and building partnerships among health professionals, families, and communities to improve the nutrition status of infants and toddlers.

What do families need to do to promote the nutrition status of infants and toddlers?

♦ **How can you become more effective in promoting the nutrition status of infants and toddlers?**

Legal Responsibilities

Standards for the Licensure or Approval of Family Child Care; Small Group and School Age and Large Group and School Age Child Care Programs state in section 7.12 the following requirements apply to all delivery models of childcare. I included the most relevant for you to read that is why the numbers and letters skip.

- (a) in USDA recognized nutrition requirements for the healthy growth and development of children; and
- (b) in food choking hazards.
- (a) the nutrition program must serve a variety of nutritious foods;
- (b) the program must ensure that snacks and meals are prepared and served in a safe and sanitary manner;
- In programs serving infants and toddlers:
 - A current feeding schedule must be maintained, documenting use of either breast milk or formula, if applicable, new foods introduced, food intolerances and preferences, voiding patterns, and observations related to developmental changes in feeding and nutrition.
 - Infants and toddlers must be fed according to their individual feeding schedules or needs.
- 3. Infants must be held while fed a bottle.

- The program must offer a nutritious substitute to a child who misses a meal or snack due to deviations from his regular schedule (such as arriving late or leaving early).
 - (a) be present interacting with and assisting children;
 - (b) allow children to eat at a reasonable, leisurely rate;
 - (c) encourage children to serve themselves, when appropriate;
 - (d) ensure that each child receives an adequate amount and variety of food;
 - (e) help children with disabilities to participate in meal and snack times with their peers;
 - (f) encourage children to eat a well-balanced diet;
 - (g) offer alternative activities for children who have finished their snack or meal.

Training Options

1. The [United States Department of Agriculture](#) (USDA) has compiled food safety and general nutrition information targeted at Child Care Educators
2. Menu Planning for Child Care
3. Choking Prevention

What did you discover about your responsibilities as an educator for nutrition by reviewing the regulations and exploring the resources?

Let's Move

[Let's Move! Child Care](#) a part of the initiative launched by Michelle Obama dedicated to solving the obesity problem. Let's Move! is about putting children on the path to a healthy future during their earliest months and years. **What idea might you use from Let's Move Childcare?**

Read and Participate

CRITERIA	NOTES	EXAMPLES FROM WEEKLY CONTENT
Connections		
Implementation		
Curiosities Why?		
Additional Online Resource(s) to Share		

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1.5: Developmental Influences- Health

HEALTH

Learning Objective

The objective in this section is for you to explain how health influences growth and development This meets the NAEYC Standard 1b, 6b and the MA Core Competency 1.A. 2 @ Initial & 8.D.12@Initial level.

[NAEYC Code of Ethical Conduct and Statement of Commitment](#) (2005) the primary responsibility of an educator is to provide care and education in settings that are safe, **healthy**, nurturing, and responsive for each child. The first principle to guide our conduct and ethical responsibility to children is

P-1.1—Above all, we shall not harm children. We shall not participate in practices that are emotionally damaging, physically harmful, disrespectful, degrading, dangerous, exploitative, or intimidating to children. This principle has precedence over all others in this Code.

- Separate the diapering and toileting areas from food preparation and feeding areas.
- Keep all areas clean.
- Wash hands frequently
- Provide easy to clean and suitable surfaces and covering for the activities in the area
- Heat, light, ventilation, and acoustics impact health. Natural lighting, soft elements to reduce noise, and screened, openable windows are ways to improve health.
- 1. Abuse and Neglect
 2. Lead poisoning
 3. Asthma
 4. Unintentional Injuries
 5. Oral Health

Abuse and Neglect

Chapter 2 Toddler Development Review [The Youngest Americans: A Statistical Portrait of Infants and Toddlers in the United States](#) to read the statistics for the number of infants and toddlers that suffer abuse and neglect. How does the the abuse and neglect of infant and toddlers compare to other age groups? What are the risk factors for child abuse and neglect? What are your responsibilities for reporting?

Lead poisoning

Preventing lead poisoning in young children Atlanta: CDC; 2005.

Protecting Children from Lead Poisoning (2014)

Asthma

Resource to Review

aring for Kids with Asthma: A Guide for Massachusetts Child Care Programs (2013) **What are your experiences with asthma? How will you use the Asthma Action Plan (AAP) and individual health care plan (IHCP) described in the document**

Unintentional Injuries

^[1] (2006) identified the injuries most common for each age group of children. For infants, suffocation and falls are the most common. For toddlers, falls and poisonings are the leading causes of nonfatal injuries, followed by scald burns and motor vehicle-related injuries. There are many regulations designed to prevent injury such as regular building inspections, use of guardrails and protective barriers, safety covers for electrical outlets, requirement for an integrated pest management policy, use and storage of toxic substances, fire and carbon monoxide detectors, CPR and first aid training, and the use of injury reports. **How many of these practices have you seen in use?**

What about toddlers make them accident prone?

Infant Toddler Foundations (2008).

Young toddlers may begin to:

- Experience cause and effect (going downhill fast cause falls; turning the TV up too loud hurts ears).
- Show some caution on uneven ground and heights.
- Notice and imitate adult reactions to dangerous people and situations.
- Respond to warnings and directions from others.
- Understand the difference between what should be eaten and what should not.

Older toddlers may begin to:

- Remember cause and effect experiences and apply their experiences to future situations (avoid touching cold railing, walk slowly down hill where fall happened).
- Increase self-control over their impulses.
- Recognize and avoid situations that might be unsafe.
- Understand what their bodies can do and understand their limits.
- Watch for adult reactions to unfamiliar things or situations that might be dangerous.
- Understand and follow basic health and safety rules. They still require close supervision from caregivers to follow these health and safety rules consistently.
- Feel proud when they follow safety rules and ashamed when they do not (say, “Look, I waited!” at corner; hang head after trying to reach forbidden item).

Oral Health

A Healthy Mouth for Your Baby . This 4 minute video is for families of infants or toddlers. It explains why baby teeth are important, gives tips on how to prevent early childhood tooth decay, and promotes a dental visit around 12 months. (2013)

The regulations require

The children are in care for more than 4 hours, or they have a meal while in care.

1. Use a small smear of fluoride toothpaste
2. Spit it all out in the sink
3. NOT rinse with water after brushing.

How will you support healthy oral health habits?

Growing Healthy Smiles In The Child Care Setting: Implementing a Tooth Brushing Program to Promote Oral Health and Prevent Tooth Decay. Boston: Massachusetts Department of Public Health.

Is the environment healthy for infant and toddlers?

[Let's Move! Child Care Checklist Quiz](#). The Let's Move! Child Care best practices are listed on the left. Please check the box under the statement that best describes the current situation of the program.

Eco-Healthy Child Care® Checklist developed by the Children's Environmental Health Network. The checklist reflects best practices for health as described in the Caring for Our Children: National Health and Safety Performance Standards, 3rd Edition. By completing the checklist you will discover 30 ways to benefit the health and well-being of the children in childcare program.

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1.6: Developmental Influences- Socio-Economic Status

SOCIO-ECONOMIC STATUS

Learning Objective

The Youngest Americans: A Statistical Portrait of Infants and Toddlers in the United States by Child Trends. Publication #2013-48, many infants and toddlers are beginning life with limited financial resources. The report presents a profile of children from birth through age eight years and includes information about their health status, access to educational and social service programs, and their families. Some of the statistics in the report include:

- 48% of America's infants and toddlers live in low-income families; 25% live in families below the official poverty line; 13% are in deep poverty
- Economic disadvantage is concentrated in the families of black and Latino infants and toddlers; 66% of these young children are in low-income families.
- 24% of black and Latino infants and toddlers live in households that are "food-insecure" (p.1)
- 24% are raised by a single parent
- 16% are in the care of grandparents

SHORT TERM RISK FACTORS

- Experience decreased general health with an increase in death rate from an infectious disease
- Are vaccinated at lower rates
- Have increased rates of asthma and anemia.
- Live with increased rates of parental depression which could lead to risk for abuse and neglect
- Have an increased exposure to alcohol and substance abuse.
- Need safe play spaces
- Have a greater exposure to lead
- Live in communities with an increased exposure to community and interpersonal violence.

LONG TERM RISK FACTORS

- More than 1/3 of children from low- income communities enter kindergarten classes already behind their peers and by fourth grade, more than 50% of these children will not meet the standard for reading proficiency. (Connecticut Commission on Children, 2004)
- Children living below the poverty threshold are 1.3 times more likely than non-poor children to experience learning disabilities and developmental delays. (Brooks-Dunn & Duncan, 1997)
- Low-income children are nearly 50% more likely to become obese adults than children with more financial resources. (Duncan, Ziol-Guest, & Kahl, 2010)
- Early experience affects brain development and lays the foundation for children's development. ([Harvard's Center for the Developing Child](#))

Resiliency

Coping with Early Adversity and Mitigating its Effects—Core Story: Resilience produced by the Center for Advanced Studies in Child Welfare. The video is described as:

Children are incredibly resilient – the same rapid brain development that occurs in the first few years of life that make young children particularly vulnerable to maltreatment also make those same children particularly receptive to intervention. The concept of resiliency is especially important to keep in mind when working with young children in foster care. This video features experts discussing the process that results in resiliency in young children and the factors that can be put in place to bolster resiliency.

<http://caschw.umn.edu>

Institute of Child Development: <http://www.cehd.umn.edu/icd/>

Center for Early Education and Development: www.cehd.umn.edu/ceed/

Explore

Jiang, Y., Granja, M.R., & Koball, H. (2017). [Basic Facts about Low-Income Children: Children under 3 Years](#), 2015. New York: National Center for Children in Poverty, Columbia University Mailman School of Public Health.

Murphey, D., Cooper, M. and Forry, N. (2013). [The Youngest Americans: A Statistical Portrait of Infants and Toddlers in the United States](#). Bethesda, MD: Child Trends. Publication #2013-48.

Zero to Three. (2012) [National baby facts](#) . Washington, DC: National Center for Infants, Toddlers, and Families.

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1.7: Developmental Influences- Engaging Families

Learning Objectives

- Explain how family styles influence growth and development. (NAEYC Standard 1b, 6b; MA Core Competency 1.A.2 @ Initial, 3. A. 2 Advanced. & 8.D.12@Initial)
- Compare how development is impacted by the family and community socioeconomics, values, and traditions. (NAEYC Standard 2a; MA Core Competency 3.A.1 @ Initial & 8.E.17 @ Initial)

Introduction

Do you remember learning about parenting styles in your Child Development course? The chart below may assist you in recalling the information. The first part of the chapter will look at how parenting styles impact an infant and toddler's development.

	Supportive Parent is accepting and child-centered	Unsupportive Parent is rejecting and parent-centered
Demanding Parent expects much of child	Authoritative Parenting Relationship is reciprocal, responsive; high in bidirectional communication	Authoritarian Parenting Relationship is controlling, power-assertive; high in unidirectional communication
Undemanding Parent expects little of child	Permissive Parenting Relationship is indulgent; low in control attempts	Rejecting-Neglecting Parenting Relationship is rejecting or neglecting; uninvolved

From mrslocomb.wikispaces.com/Parenting+Styles

Parenting Styles

^[1]). For example, a families working conditions will impact child development. Professional families typically have more complex work schedules and tasks while families employed in factories or stores have more fixed schedules and tasks. Talib, Mohamad, and Mamat (2011) suggest that the type of parental work can result in values that get passed down and enforced to their children. Professional workers often value self-direction: freedom, individuality, creativity, where members of the working class may value conformity: orderliness, neatness, and obedience.



Effects of Parenting Styles on Children mrslocomb.wikispaces.com/Parenting+Styles

What is a typical family?

Time magazine article. "The big question is: What is replacing it? Now a new study suggests that nothing is — or rather, that a whole grab bag of family arrangements are. More Americans are in families in which both parents work outside the home than in

any other sort, but even so, that's still only about a third."

The Family: Diversity, Inequality, and Social Change. Cohen identifies the three biggest changes in family life in the past 50 years are the:

1. Decline of marriage (in 2010, 45% of households were headed by a married couple, whereas in 1960 it was close to 66%) increasing people living alone and in unrelated people living together
2. Rise of the number of women in the paid workforce
3. Multitude of family structures such as: blended, remarried and co-habiting families. Families headed by single moms, whether divorced, widowed or never married, are now almost as numerous as families that have a stay-at-home mom and a breadwinner dad — about 22% and 23%, respectively.

Source: "There is No Longer Any Such Thing as a Typical Family," by Belinda Luscombe, *Time*, September 3, 2014.

Family Engagement

[2]; Weiss^[3], Caspe, & Lopez, 2006). As a means to supporting family engagement and children's learning, it is crucial that programs implement strategies for developing partnerships with families (Henderson & Mapp, 2002^[4]). The first reading [Family Involvement Makes a Difference: Family Involvement in Early Childhood Education](#), from the [Harvard Family Research Project](#) reports on the research and provides many research based strategies for you to consider in your work with children and families.

[Division for Early Childhood](#). The position statement defines responsive behavior. The National Association for the Education of Young Children NAEYC created a comprehensive definition of family engagement that features six factors:

1. Families should act as advocates for their children and early childhood education program by actively taking part in decision making opportunities.
2. Consistent, two-way communication is facilitated through multiple forms and is responsive to the linguistic preference of the family.
3. Families and early childhood education programs collaborate and exchange knowledge.
4. Early childhood education programs and families place an emphasis on sustaining learning activities at home and in the community.
5. Programs and families collaborate in establishing goals for children both at home and at school.
6. Early childhood education programs create an ongoing and comprehensive system for promoting family engagement.

Gonzalez-Mena^[5] illustrates responsive family engagement with several examples. **How will you suspend judgment long enough to try to understand a different perspective that families may have? How will you respond if the differences are harmful to children? How far can childrearing practices be respected while at the same time ensuring children's rights are protected?**

Gillespie^[6] describes specific techniques to foster positive family engagement. **Why should you develop positive relationships? What techniques of family engagement would you like to develop?**

Read and Participate

Reading Reflection Form

CRITERIA	NOTES	EXAMPLES FROM WEEKLY CONTENT
Connections		
Implementation		
Curiosities Why?		
Additional Online Resource(s) to Share		

References

1. Talib, J., Mohamad, Z., & Mamat, M. (2011, May). Effects of Parenting Style on Children Development. *World Journal of Social Sciences*, 1(2), 14-35. [↩](#)
2. Henrich, C. & Gadaire, D. (2008). Head Start and parental involvement. *Infants and Young Children*, Vol. 21 (1), pp. 56-69. [↩](#)

3. Weiss, H. B., Caspe, M., & Lopez, M. E. (2006). Family involvement in early childhood education. Cambridge, MA: Harvard Family Research Project. [↵](#)
4. Henderson, A. T., & Mapp, K. L. (2002). A new wave of evidence: The impact of school, family, and community connections on student achievement. Austin, TX: National Center for Family & Community Connections with Schools Southwest Educational Development Laboratory. [↵](#)
5. Gonzalez-Mena, J. (2010). Cultural responsiveness and routines: When center and home don't match. *Exchange* (19460406), (194), 42-44. [↵](#)
6. Gillespie, L. G. (2006, September). Cultivating Good Relationships with Families Can Make Hard Times Easier! *Young Children*, 53-55 [↵](#)

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1.8: Professional Resources and Organizations

Learning Objectives

- Identify, locate, evaluate, and responsibly use and share that information to explain various factors such as how culture, nutrition, health, socio-economic status, and family styles influence growth and development. (NAEYC Standard 1b, 6b; MA Core Competency 1.A.2 @ Initial & 8.D.12@Initial)
- Compare how development is impacted by the family and community socioeconomics, values, and traditions. (NAEYC Standard 2a; MA Core Competency 3.A.1 @ Initial & 8.E.17 @ Initial)

Introduction

What is the problem at hand? In the context of this course, the “problem” is to determine “evidence-based professional practice for infant and toddler education and care. More specifically you will create a list of resources to answer one of the course essential questions:

1. How do relationships influence infant and toddler development and learning? Why are interactions essential?
2. What is the “curriculum” for infants and toddlers?
3. How should families be engaged as collaborative partners”? Why is it essential to engage families in developing and implementing individualized curriculum for infants and toddlers? How is this achieved?

What is and why should I use research?

NAEYC provides guidelines or using Early Childhood Research to support your teaching practice. Research provides information to make decisions on behalf of young children and families. So, consumers of research must be well-informed. NAEYC suggests using peer reviewed research to:

- Answer an important question that can be investigated through experiences.
- Build on relevant theory and previous research, as seen in detailed references to others’ work.
- Present an independent, balanced, and objective approach.
- Evaluate alternative explanations for the findings.

Online Guidelines and Resources are available to help early childhood professionals become well-informed consumers of research. Explore how to assess research quality and use research for decision making.

Ethical Standards for Research are critically important when conducting research with young children and other vulnerable populations. Resource links provide information about:

- Research procedures must never harm children, physically or psychologically.
- Children and their families have the right to full information about the research in which they may participate, including possible risks and benefits. Their decision to participate must be based on what is called “informed consent.”
- All those who receive federal funds for research must use specific informed consent procedures with research participants.
- Children’s questions about the research should be answered in ways children can understand.
- Children and their families have the right to refuse to participate in research or to withdraw from participating at any time.
- Information obtained through research with children should remain confidential. Researchers should not disclose personal information or the identity of participants in written or oral reports and discussions.

primary source is a report of an original research study. A primary source usually provides enough details to replicate the research study. Primary sources are written by the researcher(s) or evaluator(s) who conducted the study. The main formats of primary sources are journal articles, technical reports from research institutions or education organizations, and reports on presentations at conferences.

secondary source is a description and summary of one or more prior research studies. Secondary sources usually do not include enough details to replicate the original studies being described. Examples of secondary sources are literature reviews and books. Although newspaper articles also can be secondary sources, they often do not have enough information to help readers form a solid judgment about the research. Essays by education experts can be secondary sources of education research, but essays can be overly biased toward the views of the writer.

Finding Education Research

NAEYC's Office of Applied Research bridges the gap between early childhood research and practice. You can access to high-quality research in order to ensure the practices you use in the classroom are best for the children and families you serve. At this website you can access early childhood research.

Below is a list of additional websites to use when searching for information.

THE EDUCATIONAL RESOURCES INFORMATION CENTER (ERIC)

www.eric.ed.gov. The following terms will produce search results:

- Infant Behavior
- Infant Care
- Infant Mortality
- Infants
- Premature Infants
- Young Children
- Toddlers
- Developmentally Appropriate Practices
- Early Childhood Education
- Early Experience

BRIDGEWATER STATE UNIVERSITY LIBRARY

[ERIC](#), [Education Research Complete](#), [Academic Search Premier](#), [PsychInfo](#), and many others.

SEARCHING THE WORLD WIDE WEB

RETAINING, ORGANIZING, STORING, AND REUSING RESOURCES

^[1] explains social bookmarking as the recording and saving of content for later use and quick access. It is a “collaborative and collective saving and sharing of web content”. (¶1)

- ANYONE can publish on the Internet! Can you distinguish between a 7th grader and Johns Hopkins University?
- For *any* source, The University of California Berkeley Library site suggests considering:
 1. **Authority** – Who is the author? What is their point of view?
 2. **Purpose** – Why was the source created? Who is the intended audience?
 3. **Publication & format** – Where was it published? In what medium?
 4. **Relevance** – How is it relevant to your research? What is its scope?
 5. **Date of publication** – When was it written? Has it been updated?
 6. **Documentation** – Did they cite their sources? Who did they cite?

Student Additions to the Chapter

Local resources to support children and families of infants and toddlers

Healthy Families. (2017). Retrieved October 24, 2017, from www.kdc.org/services/healthy-families/

30 Ansel Hallet Rd, West Yarmouth, MA 02673 (508) 385-6019

The Kennedy Donovan Center (KDC) has a family support center in West Yarmouth on Cape Cod which offers an array of services for families. The Family Support Center is an important resource for families of people with disabilities. The center offers six specific service options:

1. Information and referral
2. Service navigation
3. Family trainings
4. Parent networking
5. Community connection and resources
6. Administration of flexible funding

Submitted by Megan McManus, October, 2017.

Family Resource Center Cape Cod[Pamphlet]. (2017). MA: Cape cod neighborhood Support Coalilition. 29 Bassett Lane • Hyannis, MA 02601 • 508-815-5100 Retrieved from capecodfamilyresourcecenter.org

This is a free monthly newsletter for families. The newsletter gives them list of activities that are going on in the month that they might find fun and provide support for the families who may be able to meet other families and connect with them. For families with infants and toddlers the music and movement activity is most relevant.

Some of the activities are free and some have a fee attached to them. The whole family is invited to every event some are geared towards mom's or dads or both but the child is always allowed to come. This is a great way to meet people in your community with common interests. *Submitted by Megan McManus, October, 2017.*

McLeod, C. (n.d.). Health Insurance . Retrieved October 23, 2017, from www.falmouthservicecenter.org/health-insurance.html

Falmouth Service Center is a wonderful place. They help in all kinds of ways. They have free meals twice a month. The food pantry services the people of Falmouth. However, every town has a food pantry that people can go to for food help. The center also helps people with financial assistance by helping to pay bills such as heating or rent.

Falmouth Service Center helps with health insurance too. They will help you choose the right plan for your family. "Children can get MassHealth even if their parents do not have social security numbers or a green card. Your premium costs are based on your income and the health plan you choose". Their mission is "to ease stress, reduce hunger and improve the quality of life for our neighbors in need". **The address** for the Falmouth Service Center is 611 Gifford Street, Falmouth, MA 02540. **The telephone number** is 508-548-2794. *Submitted by Samantha Para, October, 2017.*

Justice Resource Institute. (2017). JRI Leaders in social justice. Retrieved October 21, 2017, from <https://jri.org/>

For services on Cape Cod Contact:

**Amy Sypher, MA Program Development Specialist
35 Summer Street
Taunton, MA 02780**

Justice Resource Institute (JRI) is an organization meeting the needs of underserved individuals, and families since 1973. Cape Cod Child Development (CCCD) has had JRI come out and look at children who seem to need their support. They have locations in Massachusetts, Connecticut, and Rhode Island. They meet the needs of; developmentally delayed individuals living with mental illness and behavioral challenges, court-involved children and adolescents; LBGT, and children and adults who have experienced trauma. JRI also provides home-based counseling and trauma-informed care. JRI provides many services for children and their families such as, foster care services, adoption supports, and healthy after-school supports. They provide families with environments of safety and care.

JRI came to a training at our program to talk briefly about what they do. They provide and support families with all diverse types of situations. They help everyone as much as they can. Their main site is in Needham Massachusetts. They will provide services for as long as you need it, and the services that they do provide could be long-term or short-term. JRI does not just work with certain people and families, they work with everyone and anyone who needs their help. *Submitted by Danielle Faria, October, 2017*

Erikson Institute. (n.d.). Fussy Baby Network. Retrieved October 24, 2017, from www.erikson.edu/fussybaby/services/

Phone number for anytime advice: 1-888-431-BABY

Boston Medical Center: (617) 414-4767

Address: One Boston Medical Center Pl, Boston, MA 02118

The Fussy Baby Network and Erikson Institute are in Chicago and were started in 2003. It's a program that helps families who are struggling with the first year of the baby's life. This can include support for feeding, sleeping, and crying. The approach is called FAN which is Facilitating Attuned Interactions which focuses on families concerns about their child. A family wants someone to listen and to find support. You can call Monday through Friday 9 am to 5 pm. There is no charge for calling.

Fussy Baby Network services are integrated into the Healthy Steps program at Boston Medical Center. Services include phone consultation, clinic consultation, and home visitation through the primary care clinic for families. See the contact information above.

Fussy Baby Network FAN is infused into several key programs within the Jewish Family & Children's Service Center for Early Relationship Support (CERS). For more information call (781) 693-5652 or visit the [Jewish Family & Children's Service website](#). Submitted by Morgan Evans, October, 2017

[Post your hypothesis comments here](#)

[Information about developmental milestones](#)

Massachusetts Department of Early Education and Care. (2010, November). *Massachusetts early learning guidelines for infants and toddlers*. Boston, MA: Author

This is a tool that teachers as well as families can access. The learning standards offer a breakdown of what the early milestones are for children birth to 33 months of age. They have a break down into areas of development and what the child should be doing by certain ages.

This is nice for teachers to use to gauge where a child should be and what they can do to help this child. There are suggestions in each area for activities to try with the child. Submitted by Megan McManus, October, 2017.

Teaching Strategies, Inc. (2016). GOLD®. Bethesda, MD: Teaching Strategies.GOLD®. Retrieved October 24, 2017, from <https://teachingstrategies.com/solutions/assess/gold/>

"The authentic, ongoing, observation-based assessment system that helps teachers and administrators like you focus on what matters most for children's success. Grounded in our 38 research-based objectives for development and learning, GOLD® supports effective teaching and assessment, while providing you with more time to spend with the children in your program. Accessed through MyTeachingStrategies™, GOLD® automatically links teaching and assessment, making it easier to connect the dots across the most important aspects of high-quality early childhood education."

This is a company that provides teachers with assessment tools that may be used in a toddler or preschool classroom to assess if a child is meeting their milestones. This comes as a packet that may download or receive in the mail. Teachers can use this to create a profile for each child and answer the questions that are in the assessment. These can then be shared with the family as well. These can be used in parent teacher meeting to show where the child stands in the milestones. Submitted by Megan McManus, October, 2017.

Centers for Disease Control and Prevention (2016). *Developmental Milestones*. Retrieved October 23, 2017, from <https://www.cdc.gov/ncbddd/actearly/milestones/index.html>

Developmental milestones happen as a child grows and include walking, talking and even smiling. The Centers for Disease Control has tools to help families know what to look for at each stage of life. The website also has categories for example, social and emotional, and language/communication and examples to know what to look for.

If you are concerned about your child, the Center for Disease Control has a page of tips you can do to help your child, whether it's asking for a referral or getting an evaluation. The website also has a way to help the caregiver know what to say when they are asking for help. For example, when you call your child's doctor's office, say, "I would like to make an appointment to see the doctor because I am concerned about my child's development". Also, "be ready to share your specific concerns about your child when you call. If you wrote down notes about your concerns, keep them. Your notes will be helpful during your visit with the doctor". Submitted by Samantha Para, October, 2017.

The Center for Neurological and Neurodevelopmental Health. (2015, January 18). *Track your child's Developmental Milestones*.

This brochure is a visual of the developmental milestones infant-toddlers should reach at certain ages. This tracks infant-toddlers from the ages of 6 months to 4 years. This brochure has just the right amount of information to keep it clear and concise. Different components on the brochure gives you more information on what to expected with each developmental milestone, and the possible delays that come with it. It also gives a few websites, and telephone numbers in case you see any delays in your child's milestones, and want to contact someone. Also, the websites will allow you to get a little more information regarding exactly what you need.

The infant and toddler teachers and the families can learn about more diverse ways to work with any delays in meeting developmental milestones. They will mostly get services such as early intervention. Having these services will help the families get activities and plans set in place for those children. Submitted by Danielle Faria, October, 2017

Prath, S. (2016). Red flags for speech-language impairment in bilingual children: Differentiate disability from disorder by understanding common developmental milestones. *ASHA Leader*, 21(11), 32-33. doi:10.1044/leader.SCM.21112016.32

Speech development in bilingual children is sometimes over looked. The article starts off with a hypothetical situation where you get an email stating that there is a bilingual student struggling to communicate. Does this child have a speech deficit or is learning a second language causing the difficulties? Well, we shouldn't be looking at the differences of the languages, but rather what they have in common. A child should be able to reach milestones with at least one language even if it's their first language isn't English. If the child is able to reach the milestones in at least one of the languages that they speak they are achieving milestones.

Milestones include single words, two-word phrases, and vocabulary development. *"If we evaluate vocabulary development using the same language between monolingual and bilingual children- Bilingual children might seem like they understand fewer words"*. If the child can't comprehend in one or both of the languages, then a bilingual evaluation should be in place.

The article has many charts for speech development milestones, speech intelligibility milestones (for family understanding), a chart for school-age children who don't reach the milestones for language, when to contact Speech and Language Professional's for evaluation, and typical vocabulary development.

Speech development in bilingual children is a milestone that is sometimes over looked. The article starts off with a hypothetical situation where you get an email stating that there is a bilingual student struggling to communicate. Does this child have a speech deficit or is learning a second language causing the difficulties? Well, we shouldn't be looking at the differences of the languages, but rather what they have in common. A child should be able to reach milestones with at least one language even if it's their first language isn't English. If the child is able to reach the milestones in at least one of the languages that they speak or even both the child is demonstrating milestones.

These milestones include single words, two-word phrases, and vocabulary development. *"If we evaluate vocabulary development using the same language between monolingual and bilingual children- England to English, bilingual children might seem like they understand fewer words"*. If the child can't in one or both of the languages, then a bilingual evaluation should be in place. Side note, the article depicts many charts one for speech development milestones, speech intelligibility milestones (for parents understanding), and a chart if school-age children don't reach the following milestones for language, SLP's should consider evaluation, and typical vocabulary development. *Submitted by Morgan Evans, October, 2017*

[Post your hypothesis comments here](#)

[Information that promotes physical health and well-being](#)

Women, Infants, & Children (WIC) Nutrition Program . (2017). Retrieved October 24, 2017, from <https://www.mass.gov/women-infants-c...rition-program>

Main site 1019 Iyanough Road, Hyannis, MA 02601 (508) 771-7896

Women, Infants, & Children (WIC) is a free nutrition program that aims to keep Massachusetts families healthy. WIC offers food services for families who may need assistance with buying food for their families. They offer service for pregnant and breastfeeding women and children under the age of five.

If you are pregnant or have a child under 5 years old in your household, the WIC program may be able to provide you with:

- Free, healthy food
- Nutrition, breast feeding support and education
- Referrals for medical and dental care, health insurance, child care, housing, and fuel assistance

You must apply and qualify before using their services. You can participate in WIC if you:

- Live in Massachusetts
- Have a nutritional need (WIC staff can help you determine this)
- Are a child under 5, a new mom, or a pregnant or breastfeeding woman
- Have a family income less than WIC guidelines. You are automatically income eligible for WIC if you currently receive:
 - MassHealth/Medicaid insurance plans
 - Supplemental Nutrition Assistance Program (SNAP)
 - Transitional Aid to Families with Dependent Children (TAFDC) or cash assistance

This is a great way for families to buy healthy food options for their children. *Submitted by Megan McManus, October, 2017.*

Early Intervention Parent Leadership Project (2013). Basic Information on EI. Retrieved November 1, 2017, from eiplp.org/basic-information/

If you think your child has a development issue, and the child is three or younger then Early Childhood Intervention can help. If your family is eligible for services, the family will be assigned a provider who will work with the family and early intervention team to develop and carry out an individualized service plan that addresses the child's developmental needs and the family's priorities. There are many certified community-based programs serving all cities and towns in Massachusetts. "Each Early Intervention program is certified to provide services for a specified group of cities and towns, called a catchment area". The service is free to families!

Early Intervention helps the child in the socially and emotionally too. The adults that work with the child create a bond with them. The children know they are safe and a positive relationship can. There are different activities the Early Intervention staff does with the children to help them. For example, simply talking to the child and asking them how they are feeling. Even being silly and making the child laugh is fostering a positive and rewarding relationship. *Submitted by Samantha Para, October, 2017.*

[Post your hypothesis comments here](#)

[Information that promotes social and emotional health including Promoting positive relationships and reducing child abuse or neglect](#)

Zero to Three. (2010, February 21). *Tips on Helping Your Child Build Relationships*. Retrieved October 31, 2017, from <https://www.zerotothree.org/resources/227-tips-on-helping-your-child-build-relationships>

Positive relationships to be are extremely important in a child's life. Having a role model or someone you can look up to is a way to stay safe. Just as stated earlier in this book if the child has an interested and caring person by their side, that child will be resilient.

There is a list of strategies to help a child build positive relationships. First, allow for unstructured, uninterrupted time with you're the child each day. Play with your child and be engaging with them. Next is, say things like, "You are using so many beautiful colors to make that drawing" Respect your child's feelings. Provide opportunities for your child to develop relationships with peers, so they have a lot of practice to understand to take turns, share, problem solve, and feel the joy of friendship. Next limit "Screen Time as it limits the bonding time with the caregiver and experiencing the world. If the child does have screen time, make it beneficial by asking questions about the show. For example, how it made them feel or what was your favorite part. *Submitted by Samantha Para, October, 2017.*

Wang, K. (2013, November 08). 8 Important Tips for Working with A Special Needs Child – Friendship Circle – Special Needs Blog. Retrieved November 01, 2017, from [http:// www.friendshipcircle.org/blog/2012/10/15/8-important-tips-for-working-with-a-special-needs-child/](http://www.friendshipcircle.org/blog/2012/10/15/8-important-tips-for-working-with-a-special-needs-child/) The population of children with special needs is growing and more adults, teachers, and coaches, might be finding themselves working with children with special needs for the first time. It's important to build relationships with children. Some people who are working with children with special needs are either professionals or volunteers. Wang offers 8 tips in order to work with children with special needs interact, observe, use common sense, be flexible, be consistent, use visual, auditory or tactile cues, have a plan and backup plan, and be positive.

To interact, introduce yourself to the child and explain why you are connecting with this child. By observing you are looking out for differences in behavior and how the child communicates. If you are unsure of something, always ask their families. Using common sense, always make sure that the environment that the child is learning in is physically and emotionally safe and comfortable for them. Flexibility means to accommodate for the child; some adults are not willing to accommodate for the child; the key is to have a variety of methods to help the child. Consistency is as seen in a set of rules, make sure that if there is a set of rules, they are consistent to everyone. Visual, auditory or and tactile cues that are helpful to promote a child's participation. Sometimes you need a plan B if your first plan isn't working, there's nothing wrong with that, but make sure there is a space for the child to calm down if they react in a way that they don't like. Most importantly be positive, a positive attitude can really contribute to a child's confidence.

The population of children with special needs is growing and with that, more and more adults, teachers, and coaches, might be finding themselves working with these children for their very first time. It's important to build relationships with these children. Some people who are working with these children are either professionals or they are volunteers. Wang talks about 8 tips in order

to work with children with special needs. These 8 tips are to interact, observe, use common sense, be flexible, be consistent, use visual, auditory or tactile cues, have a plan and backup plan, and be positive.

To interact, means you introduce yourself to the child and explain why you are connecting with this child. By observing you are looking out for differences in behavior and how the child communicates with you. If you are unsure of something, always ask their parents. When using common sense, always use safety first, make sure that the environment that the child is learning in is physically and emotionally comfortable for them. Flexibility means to accommodate for the child. Wang stats that some adults are not willing to accommodate for the child, the key is to have a variety of methods to help the child. Consistency is important for set of rules, make sure that if there is a set of rules, they are consistent to everyone. Visual, auditory or tactile cues are helpful when used in an environment, it can even be the difference between a child's participation and non-participation (Wang, 2012). Sometimes you need a plan B if your first plan isn't working, there's nothing wrong with that, but make sure there is a space for the child to calm down if they react in a way that they don't like. Most importantly be positive, a positive attitude can really contribute to a child's confidence. *Submitted by Morgan Evans, October, 2017*

[Post your hypothesis comments here](#)

[Information that promotes stimulating language and communication](#)

Mendelsohn, A. L., Brockmeyer, C. A., Dreyer, B. P., Fierman, A. H., Berkule-Silberman, S. B., & Tomopoulos, S. (2010). Do Verbal Interactions with Infants During Electronic Media Exposure Mitigate Adverse Impacts on their Language Development as Toddlers? *Infant and Child Development*, 19(6), 577-593. doi:10.1002/icd.711

This article is about how language is effected by media including how much the young child's language can be affected; as well as later effects on the child. They studied 253 families to see how media impacts language development in their young child. They started at 6 months old and then checked back in around 14 months to see the impacts media had on the child. The findings were that media exposure was associated with reduced language development. However, this was effect was less when families talked to the infant or toddler while the child was viewing. This suggests a potential strategy for reducing negative impacts of media on young children by talking with the child about the media as they watched. *Submitted by Megan McManus, October, 2017.*

Zero to Three.(2017, February 25). In How to Support Your Child's Communication Skill.

On the website *Zero to Three*, examples of how to support the language and communication are addressed, through both verbal and nonverbal language. It Support begins with a newborn and a 9-month-old using non-verbal communication such as nuzzling at the mother's breast or using their hands to get what they need meet, then to a 28-month-olds communicate what she sees by verbal and non-verbal ways (pointing and saying the word "Derl" for squirrel). Finally, to a 3-year-old with complete verbal language telling a story about his preschool.

The website explains many ways to stimulate infant-toddler language and communication. One way is by responding to their looks, sounds, and gestures. As stated in the website, "when he puts his arms out to you, pick him up, kiss him and use simple words. "You want up". When he coos, coo back. When he gazes at you, make eye contact and talk with him". This is saying that all communication is crucial to infant-toddlers, whether it is verbal or non-verbal.

These quick responses by you tells the child that their language and communication is are essential to you, and this will encourage allow the language and communication of infant-toddlers to be endless and so they will develop their skills quicker. Secondly, is you should always talk and listen to your child. With infant and toddlers giving them the understanding that you are there to listen and talk is important for their development, because they feel like they are being heard, and will talk more. Lastly, is by narrating the daily routines. Infant and toddlers need the words as well as a visual of the word to really connect them together.

At my center in the infant-toddler room the teachers they go over what they are going to do through the day. They write on a white board and hang it outside the classroom, so when families come and pick up their child they can read the note and know exactly what their child did and learned that day. When the children arrive, the teachers have smiles on their faces and are welcoming. They say hello to the families, and get the children ready for the day. Having some not talking yet they will bang objects on the table, or do a scream or a sound to get attention. Supporting both verbal and non-verbal communication is very important for infant-toddlers. *Submitted by Danielle Faria, October, 2017.*

[Post your hypothesis comments here](#)

[Information that promotes enhancing cognitive development](#)

Guyton, G. (2011, September). In Using Toys to Support Infant-Toddler Learning and Development.

In the journal article, “Using Toys to Support Infant-Toddler Learning and Development” the author describes many different toys and activities that families can use with infant-toddlers to enhance their cognitive development. Cognitive development is how infant-toddlers develop their thinking skills.

Under the heading, “Homemade toys and readily available materials”, she mentions how many advertisements have lead people to believe that toys are better if they are store-bought or expensive, when really, materials that you have at home are superior. Guyton goes into detail how using homemade materials such as; “fabrics, bottles, cardboard boxes, yard, cooking pans, pine cones”, can be very engaging to infant-toddlers, and is an excellent way to start building relationships between the child and caregiver.

Hiding a toy under a scarf and playing the peek-a-boo game are ways teachers can enhance the cognitive skill of object permanence. Drumming on pots and pans in front of a child, will allow the child to practice imitation, and think about how to make the same sound you just made. Under the next heading, “Choosing and using toys to support cognitive development” The first object is fabric, and the many ways you can use this material, such as in dramatic play, and pulling it out of a hat. Across from the heading is an example of how a scarf is used to enhance cognitive development. An 8-month-old and a teacher put a scarf over a doll, and asks where the doll is. They lift the scarf and the doll is revealed. This is an example of object permanence. Read the rest of the article to learn about using blocks, puzzles, and rattles.

Caregivers enhance cognitive development in many ways, the first way is giving the infant-toddlers activities and games that will support and spike the curiosity of the children, such as a mystery bag or creating bubbles. Second, is to ask open-ended questions, and allowing time to the answer those questions. For example, “What do you think will happen if?” or “Can you think of another way to do this?”. Giving children time, and space and a variety of different toys and activities will allow their cognitive development to grow. *Submitted by Danielle Faria, October, 2017*

[Post your hypothesis comments here](#)

[Information that tells you how to create positive approaches to learning](#)

Zakrzewski, V. (2013, August 21). [How to Create a Positive School Climate.](#)

This article posted in Greater Good Magazine describes three easy things a teacher can do to make a positive change in the classroom. Zakrzewski describes a positive school environment, assessing the current climate, and working with others to create a shared vision for the school. When children and teachers feel valued and appreciated their classrooms will be more positive.

This article discusses and gives examples of the three R’s for infant and toddler learning. The first is **respect** which is “shown when adults treat the young child in ways similar to how they would like to be treated by others” The second is **response** this is done by observing the children and becoming sensitive to their needs. Reading body language and learning their temperaments is key to their success in school. The third is **relationship** creating a positive connection with each child; helps encourage a positive school and classroom environment.

The examples called “*Ideas into Practice!*” include what the environment looks like, establishing a primary caregiving system, and taking care of yourself. They are ideas they give for how to stay positive as a teacher. They also have some activities for the children. This article is very helpful in setting up a happy positive classroom. *Submitted by Megan McManus, October, 2017.*

Gronlund, G. (2013). [How to Support Children’s Approaches to Learning? Play with Them!](#)

Positive approaches to learning is part of successful learning experience. How can we as educators do this, is by play! Young children gain so much by playing. The children explore, learn and play with new things everyday when educators use positive approaches to learning. Simple things such as a toddler stacking rings on the post is problem solving.

If educators have a positive approach to learning it will help the children in later years in school and life. Families can have a positive approach to learning at home too. They can play with their child, interact with your child, have a conversation, or reading books to their child helps in so many ways. Even cooking together is a positive approach to learning because it helps the child bond with an adult. The child needs to have that bond with a special adult to them. Why? *Submitted by Samantha Para, October, 2017.*

The most positive approach to learning is to play with the child. To encourage a toddler to play sit on the floor with the child, bring a basket of toys with you and allow the child to be curious as to what to use. For example, using rattles the child can hear different sounds and with blocks they can stack them and then take their creation down. Problem solving is stimulated with the rings on stacking toys. Ask the child questions as they are playing.

For example, Gronlund describes a father and son playing. As the son is playing, the Dad says, “I see you are trying to get the last ring on the post, but it just won’t fit” or he asks “Where did the ball go, is it hiding behind the chair”. This type of questioning will encourage perseverance and grab the child’s attention and problem solving. *Submitted by Morgan Evans, October, 2017*

Leonard, J. (2017, September 21). Babies Can Learn the Value of Persistence by Watching Grownups Stick with a Challenge. The Conversation.

In the newspaper; The Conversation, Julia Leonard explains ways to create positive approaches to learning. She illustrates how self-control and persistence increase academic outcomes. She describes an experiment that her and her colleagues did with the infants and toddlers trying to activate a musical toy by hitting the button. The children observed either someone working hard to achieve two different goals, or someone who effortlessly reached each goal. Infants and toddlers who saw the adult persist and succeed, pushed the button about twice as many times as those who saw the adult effortlessly

Leonard concludes that persistence and positive learning comes from an adult model. Seeing this quality in the adults will allow the child to have a positive approach to learning. One way you can create a positive approach to learning is showing the infants and toddlers a box that will not open on the first try. Modeling persistence is a fantastic way to teach them a positive approach to learning. *Submitted by Danielle Faria, October, 2017*

[Post your hypothesis comments here](#)

Reviewing Resources

[Research matters: Moving to evidence based professional practice.](#) Educational Leadership, 63 (6), 87-90. You may find the table below useful when creating your annotations for the resources you will add to this chapter.

3	Things I discovered from the article and will use in teaching infants and toddlers.
2	Things I found interesting ...
1	Question I Still Have ...

Research review to add to the ECPK 324 Text

Research review to add to ECPK 324 Text. This is an opportunity to be a scholar and revise and improve the course text by creating 5 comments using **hypothesis**.

Have you signed up for hypothesis? If not go to: <https://web.hypothes.is/start/>. Create a free account. Now you add Hypothesis to your browser (Chrome, Safari, or Google). You are ready to start annotating or commenting in Chapter 8 of our textbook.

- Local resources to support children and families of infants and toddlers
- Information about developmental milestones
- Information that promotes physical health and well-being
- Information that promotes social and emotional health (promoting positive relationships and reducing child abuse or neglect)
- Information that promotes stimulating language and communication
- Information that promotes enhancing cognitive development
- Information that tells you how to create positive approaches to learning

text. Here are examples of APA for a variety of reference types.

Print

Book Example: Otto, B. (2010). *Language development in early childhood education*. (3rd Ed.) Upper Saddle River, NJ: Merrill..

Journal Article Example: Steiner, Amanda Mossman. 2011. A Strength-Based Approach to Parent Education for Children with Autism. *Journal of Positive Behavior Interventions*13(3):178–90.

Electronic General formula :

Title of Page/Document. Retrieved from URL

Note: The title of the page/document is *italicized*.

Note: n.d. indicates that no publication date is available.

Example: Public Broadcasting Service. (n.d.). *Positive ways to talk and listen.* Retrieved from www.pbs.org/parents/talkingwi...trategies.html.

Journal Article – online database: Nilsson, M., Ferholt,B., & Lecusay, R. (2017) The playing-exploring child’: Reconceptualizing the relationship between play and learning in early childhood education. *Contemporary Issues in Early Childhood*. Available at: doi-org.libservprd.bridgew.edu/10.1177/1463949117710800

Journal of Applied Psychology, 78, 443-449. Retrieved from EBSCOHost database

Web page authored by an organization

Families and Work Institute (n.d.). *Main Points from Mind in the Making.* Retrieved from: http://mindinthemaking.org/PDF/mainpoints_mitm_120426.pdf

Technical Report

A child becomes a reader. Jessup, MD: National Institute for Literacy.

Infant Toddler Guidelines

Massachusetts early learning guidelines for infants and toddlers. Boston, MA: Author.

Newspaper articles (online):

Pamphlet-Brochure

Guidelines for reporting and writing about people with disabilities (4th ed.) [Brochure]. Lawrence, KS: Author..

Sample Comments in 2 sections

Note that they are 1 and NOT the required 2 paragraphs

- Local resources to support children and families of infants and toddlers

Kindergarten Registration and Screening

Kindergarten Registration Forms. Retrieved from:

www.barnstable.k12.ma.us/Page/622

- Promoting physical health and well-being

Dental Check-up

Dental Care. Retrieved from:

chcofcapecod.org/services_dental_care.html

Grading Rubric

Criterion	Emerging Understanding	Acceptable	Target	Points

Criterion	Emerging Understanding	Acceptable	Target	Points
Variety of resources identified MA Core Competency 1.A.2 @ Initial & 8.D.12@Initial NAEYC Standard 1b	Identified less than 3 resources. Some were outdated or no date was indicated.	Identified 3-4 resources. Some were outdated or no date was indicated. The student used primarily 2 types of sources (books, journals, or websites).	Identifies 5 or more professional resources and organizations to enhance professional development and skills. The information is current (within last 10 years) and a variety of books, journals, and information from appropriate websites were explained.	50
Annotations NAEYC 6d: Integrating knowledgeable, reflective, and critical perspectives on early education	APA style was not used.	Some of the resources are fully cited using APA style. At least 4 of the annotations are 2 paragraphs that summarize the information in the resource.	The 5 resources are fully cited using APA style and are complete so that others can find them. All of the annotations are 2 paragraphs that clearly summarize the information in the resource. It is clear how the reader would use the resource to as a research base for developmentally appropriate practice & community resources.	75
Mechanics	Writer makes 3-4 errors in grammar or spelling that distract the reader from the content.	Writer makes 1-2 errors in grammar or spelling that distract the reader from the content.	Writer makes no errors in grammar or spelling that distracts the reader from the content.	25
Total				150

References

10 Excellent Social Bookmarking Tools for Teachers. Retrieved from <http://www.educatorstechnology.com/2014/02/10-excellent-social-bookmarking-tools.html>.

1. Kharbach, M. (2014, February). 10 Excellent Social Bookmarking Tools for Teachers. Retrieved from <http://www.educatorstechnology.com/2014/02/10-excellent-social-bookmarking-tools.html>. ↵

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